

COGNITIVE BEHAVIORAL THERAPY (CBT) – SESSION 1

Full Detailed Initial Assessment & CBT Model Introduction Fictional Educational Example — Highly Detailed for Training

SESSION INFORMATION

Client:	RK (35 years, Male)	Therapist:	Dr. A. (Clinical Psychologist)
Session #:	1 of 16 planned	Date:	Hypothetical – First Session
Duration:	55 minutes	Setting:	Outpatient clinic, Bangalore
Presenting Issue:	Moderate-Severe Generalized Anxiety Disorder with burnout	Referral:	Self-referred

PRE-SESSION THERAPIST PREPARATION

New client. High-achieving software engineer presenting with 10 months of excessive worry about work performance, frequent physical anxiety symptoms, sleep disturbance, irritability, and signs of burnout. He continues to perform at a high level but at significant personal cost. Self-referred after difficult feedback in a performance review. BDI-II score in waiting room = 22 (moderate depression). GAD-7 = 18 (severe anxiety). No previous psychiatric treatment or therapy. No substance use. Risk appears low at this stage. Plan for Session 1: Build rapport, gather presenting problems and history, introduce the CBT model using one of his examples, set initial collaborative goals, and assign simple self-monitoring homework.

SESSION START – BUILDING RAPPORT & SETTING THE FRAME

Therapist: Good morning, RK. Thank you for coming in today. I know that deciding to start therapy can feel like a significant step, and many people feel a mixture of nervousness and hope. How are you feeling about being here right now?

Client: A bit anxious, to be honest. I've never done anything like this before. But I've reached a point where I know I can't keep pushing through like this. My work has become really difficult, and it's affecting everything else too.

Therapist: That's completely understandable, and it's actually a very common feeling at the beginning. Many people feel both relief that they're finally doing something and nervousness about what it will involve. I want you to know that this is a safe, confidential space. Everything we discuss stays between us unless there is an immediate risk of serious harm to yourself or others. Today we'll go at your pace. I'll ask some questions to understand what's been going on for you, I'll explain how CBT works, and together we can see if this approach feels like a good fit. Does that sound okay?

Client: Yes, that sounds fine. I appreciate you explaining it clearly.

PRESENTING PROBLEMS

RK described a 10-month history of excessive, difficult-to-control worry focused primarily on his performance at work. He worries constantly about making mistakes, being judged by seniors, and eventually being “exposed” as not good enough despite objective evidence of strong performance (multiple promotions, good feedback).

Physical symptoms are prominent and almost daily: racing heart, sweating, muscle tension (especially shoulders and jaw), and occasional panic-like episodes (shortness of breath, dizziness, fear of losing control) especially before important meetings or presentations.

Sleep is significantly disturbed — difficulty falling asleep due to racing thoughts about the next day's work, and early morning awakening (around 4–5 AM) with worry. He feels constantly fatigued.

He reports increased irritability with his wife and colleagues, reduced ability to concentrate, and a growing sense of emotional numbness or “flatness” even though he continues to deliver high-quality work.

Behaviorally, he has started avoiding casual interactions with team members and has withdrawn from social activities outside work due to fear of saying something “wrong” or appearing incompetent. He often works late alone to over-prepare.

BRIEF RELEVANT HISTORY

Symptoms gradually worsened over the last 10 months coinciding with increased project pressure and a new, more demanding manager. There was one previous period of high stress during his first job (age 26) that resolved when the project ended. No history of trauma, substance use, or previous mental health treatment. Family history: Mother has long-standing anxiety treated with medication. He describes himself as a high achiever since school with perfectionistic tendencies.

INTRODUCTION TO THE CBT MODEL

Therapist: One of the core ideas in CBT is that the way we think about situations affects how we feel emotionally and physically, and then influences what we do. These three things — thoughts, feelings, and behaviors — are connected in a cycle. Would it be helpful if I showed you using one of your recent experiences at work?

Client: Yes, please.

Therapist: Can you tell me about a recent situation where your anxiety was quite high?

Client: Last week I had to present the quarterly project update to my manager and two seniors. I spent three days preparing and re-preparing the slides. The night before I barely slept.

Therapist: And in that situation, what went through your mind?

Client: I kept thinking: “If I mess up even one number or explanation, they’ll think I’m not competent. They’ll start doubting me. Eventually they’ll realize I’ve been faking it all along.”

Therapist: And when you had those thoughts, how did you feel emotionally and in your body? What did you end up doing?

Client: I felt terrified and ashamed. My heart was racing, my hands were sweating, and my mind went blank for a few seconds. I ended up staying up until 2 AM re-checking every single slide and then went to the meeting exhausted and even more anxious.

(Therapist draws a simple diagram on paper showing the cycle: Situation → Thought (“I’ll be exposed as incompetent”) → Emotion (Terror + Shame) + Physical (racing heart, sweating) → Behavior (over-prepare + avoid collaboration) → Consequence (more exhaustion and worry the next day).)

Therapist: So we can see how that one thought led to very strong feelings and then to behaviors that, while understandable, actually made the anxiety worse in the long run. CBT helps us gently examine and update these automatic thoughts so the cycle becomes less painful and more helpful. Does seeing it this way make sense to you?

Client: Yes... it actually explains a lot. I never really connected how my thinking was driving everything else.

COLLABORATIVE GOAL SETTING

Therapist: Now that we have a clearer picture, let’s think about what you would like to be different in the coming months. If therapy is successful, what would your life and work look like?

Client: I want to stop feeling terrified before every meeting. I want to be able to sleep properly. I want to feel like I can make small mistakes without it destroying my whole week. And I want to be more present with my wife instead of being mentally checked out or irritable.

Therapist: Those are excellent goals. Let’s make them specific and measurable so we can track progress. For example:

- Reduce daily worry time from several hours to under 30–45 minutes
- Improve sleep (fall asleep within 30 minutes and stay asleep)
- Feel confident enough to handle meetings without panic (even if slightly anxious)
- Reconnect with wife — have at least 3 meaningful conversations per week

Client: Yes, those feel realistic and motivating.

EXPLAINING CBT STRUCTURE & ROLES

Therapist: In CBT we usually work together for a fixed number of sessions — often between 12 and 20 for problems like yours. Each session has an agenda that we set together at the beginning. I will teach you practical skills, but you are the expert on your own life — we work as a team. Between sessions there will be small homework tasks because the real change happens when you practice the skills in your daily life. How do you feel about this active, skills-based style?

Client: I like that it’s practical and not just talking about problems. I’m willing to do the homework.

HOMEWORK ASSIGNMENT

Therapist: For this first week, I’d like you to start a simple daily self-monitoring diary. It only takes 2–3 minutes per entry. Just note:

1. Time of day and brief situation
2. Mood rating from 0 (worst) to 10 (best)
3. What you were doing at the time
4. Any strong thoughts that came up (even if they seem silly)

This will help us see clear patterns and will make our work in the next sessions much more targeted. Do you think you can manage that?

Client: Yes, I can do that.

SESSION SUMMARY & CLIENT FEEDBACK

Therapist: Today we covered quite a lot: your main difficulties, how CBT views the connection between thoughts, feelings and behaviors, your goals for therapy, and what the structure will look like. We also set your first homework. What was most useful or surprising for you in today’s session?

Client: The diagram with my presentation example really helped. I can already see how my thoughts were making everything worse. I feel a bit more hopeful that this can actually change.

Therapist: I’m glad to hear that. Any concerns or anything you’d like to do differently next time?

Client: No, this felt good. I’m looking forward to next week.

POST-SESSION THERAPIST NOTES

- Good rapport established quickly. Client engaged, articulate, and motivated.
- Clear understanding of presenting problem and its impact on work and personal life.
- Client responded very well to the CBT model introduction and could apply it to his own recent example.
- Collaborative goals set. Client expressed willingness to engage in homework.
- Risk: Low at present. Passive suicidal ideation absent. Will continue to monitor.
- Next session: Review homework diary, deepen formulation, and begin identifying key negative automatic thoughts and core beliefs.

Disclaimer: This is a completely fictional but realistic simulation of a first CBT session created solely for educational and AI training purposes. It does not represent any real person or therapy.